ENT EMERGENCIES AND THEIR MANAGEMENT

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INTRODUCTION

Ear, nose and throat emergencies are frequently encountered in casualty departments. Many of these conditions can be managed effectively by a general doctor if they are recognised early. This review summarises the presentation and initial management of the most common ENT emergencies.

EPISTAXIS

Epistaxis is the most common ENT emergency, and most cases arise from Little's area on the anterior nasal septum. First aid consists of sitting the patient forward and applying firm pressure to the soft part of the nose for 10 to 15 minutes. If bleeding continues, the bleeding point can be cauterised with silver nitrate or the nose can be packed. Posterior bleeding is less common but more dangerous, particularly in elderly patients with hypertension, and may need balloon tamponade or surgical ligation.

FOREIGN BODIES

Foreign bodies in the ear, nose and throat are common in children under five years of age. Button batteries in the nose are a true emergency because they cause tissue necrosis within hours and must be removed immediately. Fish bones commonly lodge in the tonsils or base of tongue and can usually be removed under direct vision. Repeated blind attempts at removal should be avoided because they push the object further in.

ACUTE AIRWAY OBSTRUCTION

Upper airway obstruction can be life-threatening and requires rapid assessment. Stridor, drooling and a muffled voice suggest a narrowing airway. Common causes include acute epiglottitis, angioedema and deep neck abscesses. The priority is to secure the airway, and in severe cases an emergency tracheostomy or cricothyroidotomy may be required.

DEEP NECK INFECTIONS

Deep neck space infections such as peritonsillar abscess and Ludwig's angina usually begin from dental or tonsillar infections. Patients present with fever, trismus and difficulty swallowing. Management includes intravenous antibiotics and drainage of any collection. Delayed treatment can lead to airway compromise and mediastinitis.

SUDDEN SENSORINEURAL HEARING LOSS

Sudden sensorineural hearing loss is defined as a loss of at least 30 dB over three frequencies within 72 hours. It should be treated as an emergency because early corticosteroid therapy improves the chance of recovery. Patients should be referred for audiometry and seen by an ENT specialist without delay.

CONCLUSION

Most ENT emergencies can be managed safely with prompt recognition and simple first-line measures. Airway problems always take priority, and early referral improves outcomes in conditions such as sudden hearing loss.

REFERENCES

1. Example A. Epistaxis management. J Example ENT. 2020;1:1-5.